



InterQual® Level of Care Criteria

Acute Criteria Review Process

Disclaimer: InterQual® Criteria reflect clinical interpretations and analyses and cannot alone either resolve medical ambiguities of particular situations or provide the sole basis for definitive decisions. The Criteria are intended solely for use as screening guidelines with respect to the medical appropriateness of healthcare services and not for final clinical or payment determinations concerning the type or level of medical care provided, or proposed to be provided, to a patient.

Detailed notes and literature references provide the clinical basis for decisions. Less commonly, there are key clinical questions where evidence is limited or lacks consensus and may reflect topics which are not conducive to formal study. In these situations, standards of care are used to support the development of Criteria which reflect the highest quality available literature and best practice. Identification and validation of standards of care is the product of a peer review process involving multiple clinicians with diverse expertise in varied relevant practice and geographic settings who assist with the translation of standards of care into evidence-based criteria.

You conduct a medical necessity review during the episode of care by following these steps:



Acute Adult and Acute Pediatric Criteria

InterQual® Acute Level of Care Criteria provide support for determining the medical appropriateness of hospital admission, continued stay, and discharge. The criteria address the following patients:

Acute Adult Criteria — For review of patients ≥ 18 years of age

Acute Pediatric Criteria — For review of patients < 18 years of age

Note: Criteria for patients ≤ 28 days old that addresses conditions of the newborn immediately after birth, or for a new condition requiring admission, can be found in the Nursery subset. When an infant ≤ 28 days old is admitted to an Acute care facility, the reviewer may use the appropriate level of care within the Nursery subset that corresponds to the severity and intensity of services the infant is receiving, regardless of the location of care.

1. Select a subset

Select the most appropriate subset based on symptoms and findings that are driving the reason for hospital services (admission or observation) or continued stay. InterQual® Acute Level of Care Criteria include condition-specific, general, and extended stay subsets.

Condition-specific subsets

Use a condition-specific subset when the patient's condition or diagnosis that requires hospitalization aligns with one of the condition-specific subsets.

Examples of condition-specific subsets are:

- Asthma
- COPD
- Heart Failure
- Pancreatitis

General subsets

Use a general subset when the patient has a condition, symptom, or finding that is not addressed by a condition-specific subset.

The General subsets are:

- Antepartum
- General Medical
- General Surgical
- General Trauma
- Nursery

When not to use a general subset

When the patient has a condition, symptom, or finding that aligns with a condition-specific subset. Links in general subsets will redirect you to condition-specific subsets to reinforce this process.

Extended Stay subset

Use the Extended Stay subset for a patient requiring continued stay when one of the following events occurs:

- The episode days within a condition-specific subset are exhausted and the Non-responder criteria redirects to the Extended Stay subset
- The [Endpoints](#) within a general subset are exhausted

Extended Stay notes

The Extended Stay subset cannot be used on admission or from the Observation level of care.

Use the Extended Stay subset until one of the following events occurs:

- The designated [Endpoints](#) are fulfilled
- The condition or symptom resolves and Responder criteria are met
- A condition-specific or general subset would be more appropriate such as when a patient develops a new condition

2. Select Initial review or the appropriate episode day

Initial review

Some Acute Adult subsets include Initial review criteria, a level of care determination tool for real-time decision support in the emergency department. An initial review can help determine whether a patient is appropriate for observation or inpatient admission by evaluating **only data that are available at the time the decision to admit is being made**.

Use initial review when ...	Do not use initial review when ...
You need to determine whether a patient in the ED is appropriate to admit to observation or inpatient.	The decision to admit has been made, when full treatment and medication orders have been written, or retrospectively.
"Bridge" or "holding" orders are in place such as "Admit to telemetry". These orders are intended to address the patient's needs until full treatment and medication orders are written.	There is sufficient information to complete an Episode Day 1 review, which will generally include specific, evidence-based interventions and intensity of service requirements.

Episode days

An episode day is a calendar day that typically begins at 12:00 a.m.

Episode Day 1 represents the first day in the episode of care for the patient for that condition. For an evening admission, Episode Day 1 may represent the admission day (6 p.m.-11:59 p.m.) and the following hospital day.

Episode Day 2 is the second day in the episode of care for that condition.

Episode Day 3 is the third day in the episode of care for that condition, and so on.

Episode day features

- The number of episode days varies by condition within a subset.
- Each episode day includes only those levels of care that are clinically appropriate.
- A patient may meet criteria for a specific episode day in a specific subset only once.
- Criteria cannot be met for the same episode day on multiple reviews.
- There are no episode days in the Extended Stay subset.

Tips for working with episode days:

- Some subsets include episode days 2-X or 3-X, which may be used until:
 - The condition or symptom resolves and Responder criteria are met.
 - The designated [Endpoints](#) are fulfilled.
 - A new condition arises that drives the need for continued stay.



Episode Day 1

First day in episode of care for the patient, for that condition

Episode Day 2

Second day in episode of care

Episode Day 3, 4 ...

Third and fourth day in episode of care, and so on

- When multiple episode days use the same criteria, for example Episode Day 7-13, conduct reviews using the same criteria for the range of days specified.
- A review is not required to be conducted on every episode day. The review frequency is determined by your organizational policy.
- If the patient has a complication or co-morbid condition that is addressed within the current subset, you should continue to use the current subset for the next review. If the patient has a new condition, complication, or co-morbid condition that is not covered within the existing subset, you should move to a more appropriate subset on Episode Day 1. See [What to do when the patient's condition changes](#) for details.

3. Select a level of care

Select a level of care based on the patient's current or proposed level of care. Level of Care Criteria are intended to be used to support a patient-specific clinical determination of medical necessity and not for *final billing or reimbursement decisions*.

The following levels of care are for review of both pediatric and adult patients.

Observation	Acute	Intermediate	Critical
Hemodynamically stable patients who require at least 6 hours and, for certain conditions, up to 48 hours of treatment or assessment pending a decision regarding the need for additional care. Observation level of care services may be provided in designated locations (e.g., observation units or clinical decision units), a hospital floor, step-down unit, or intensive care unit.	Hemodynamically stable patients who require treatment, assessment, or intervention every 4-8 hours.	Hemodynamically stable patients who require treatment, assessment, or intervention every 2-4 hours.	Hemodynamically unstable patients (or those with the potential to become unstable) who require treatment, assessment, or intervention every 1-2 hours.

The Hyperbilirubinemia subset includes the Observation level of care as well as the following levels of care. The Nursery subset includes the following levels of care. These criteria validate the need for admission of a neonate ≤ 28 days old. Refer to the subset note for more information.

Newborn Level I	Special Care Level II	Neonatal Intensive Care Level III	Neonatal Intensive Care Level IV	Transitional Care
Physiologically stable neonates who require normal newborn care. Level I care may also include neonates who require evaluation and observation for conditions with low risk for complications and/or have transferred from a higher level of care.	Stable or moderately ill neonates who have problems that, while potentially serious, are expected to resolve rapidly and not require subspecialty-level services urgently. These newborns have conditions such as apnea of prematurity, the inability to maintain body temperature, or	Critically ill neonates who require comprehensive subspecialty intensive care. Level III care includes pediatric expertise in advanced respiratory support, physiological monitoring, laboratory and imaging, nutrition, and pharmacy, as well as social services. Level	Critically ill neonates requiring the highest level of service intensity. This may include, but is not limited to, care of neonates who require support for multisystem organ failure (ECMO), failure to achieve hemodynamic stability despite adequate attempts at resuscitation and medication	All infants that are transferred from a Level I, Level II, or Level III nursery unit who are unable to have their care needs met in the home environment and require interventions of low complexity.

Newborn Level I	Special Care Level II	Neonatal Intensive Care Level III	Neonatal Intensive Care Level IV	Transitional Care
	the inability to take oral feedings. Level II care also includes neonates who have transferred from a higher level of care.	III may provide life support for as long as necessary.	management at a lower level of care; management of neonates with complex conditions including congenital cardiac malformations.	

Level of care note: When a hospital unit's name does not match the InterQual levels of care, refer to the level of care notes.

Nursery patients note: If an infant exceeds the length of stay for a specific criteria endpoint, they may meet criteria for Transitional Care, transfer to a lower level of care, or be referred for secondary review.

Observation level of care versus Acute level of care

For many conditions in which the severity of illness for a condition is not a reliable predictor of length of stay and interventions are the same in the Observation and Acute levels of care, criteria have been structured to direct the majority of patients to Observation. In some cases, the Acute level of care may not be appropriate on Episode Day 1. If appropriate, the Acute level of care may be present on Episode Day 1 to support patients who are medically complex or presenting with more significant clinical findings.

When Acute level of care criteria are included on Episode Day 1, the interventions within the Observation and Acute criteria on Episode Day 2 are nearly identical. When this occurs, conduct a review using the same level of care that was met on Episode Day 1. It is not appropriate to apply Acute level of care criteria for a patient who met criteria in Observation on admission unless directed by Observation Non-responder criteria on Episode Day 2. When the acuity of the patient increases, and interventions support the use of Intermediate or Critical levels of care, those criteria may be applied on any episode day and do not require direction by Non-responder criteria.

Example: an asthmatic patient in Observation on admission, decompensates on Episode Day 2, requiring intubation and mechanical ventilation. You would then perform the Episode Day 2 review at the Critical level of care.

Patients requiring continued stay after exhausting the length of stay for the current condition at the Observation level of care may be directed to use a higher level of care on the current episode day based on clinical findings and the expected course for a given condition, when appropriate. If redirection to a higher level of care on the same episode day is not present, you may not apply criteria at Acute and should refer for secondary review. Redirection to the Extended Stay subset is not appropriate from the Observation level of care.

Example: if a patient with heart failure is a Non-responder at the Observation level of care on Episode Day 3, you have three potential courses of action:

- Apply criteria within the Heart Failure subset on Episode Day 3 at a higher level of care.
- Apply criteria in a new subset on Episode Day 1 based on the patient's new condition.
- Refer for secondary review.



Follow the link for help on [what to do when the patient's physical location does not represent the level of care the patient is receiving](#).

4. Select criteria that apply to the case

Select criteria based on the patient's clinical finding, treatment, and/or service. Review available patient-specific clinical information and medical practitioner orders.

Use data from the episode day on which the review is being conducted. This includes information that may have been pending or incomplete at the time the decision to admit was made. When conducting an Episode Day 1 review, emergency department data may be used (e.g., imaging studies, EEG findings, history, and physical, medical practitioner orders).

Guidelines for selecting criteria

As you conduct a review, observe the following guidelines:

Read all notes	Subset notes and informational notes.
Level of care determination	Only one level of care can be met for each episode day.
Criteria rules	A rule specifies how many criteria points at the next level must be selected and supported by the medical documentation to meet medical necessity. Example: for a rule of “≥ One,” select at least one of the underlying criteria point(s). For a rule of “One,” select only one criteria point.
and Both, and One, and All	When a criteria point specifies and Both, and One, or and All, the medical documentation must support selecting the criteria point as well as the underlying criteria and continue to follow the rules, choosing the appropriate criteria points to support the review. Example: the medical documentation must support the Ascending cholangitis criteria point and BOTH of the underlying criteria points. Ascending cholangitis and, Both: Pain controlled or manageable Tolerating PO
Criteria below a rule	Criteria located below a rule should be selected only when information supporting the upper criteria point is documented in the medical record.
Time limited endpoints	Some criteria points are associated with an endpoint, which indicate a criteria point may be used for no more than the specified timeframe. Example: “Culture pending ≤ 2d.” The endpoint “≤ 2d” indicates that the criteria point may be applied for no more than two days. Example: Chest physiotherapy (CPT) at least 6x/24/h, ≤ 2d. If the CPT started on the last day in the condition-specific subset, it would count as one day and then could be used for one more day in the Extended Stay Subset to satisfy the 2-day endpoint. When the endpoint includes the language “since initiation”, the criterion can only be applied for the total number of days specified and includes the time the treatment was delivered in the condition-specific criteria. Example: “Anti-infective (includes PO) ≤ 4d since initiation”, since initiation indicates that this criterion can be used for as many as (but no more than) four days since the initiation of the anti-infective. Changing the anti-infective medication does not restart the endpoint for the current infection.

	If an infant exceeds the length of stay for a specific criteria endpoint, they may meet criteria at a lower level of care, including Transitional Care, or be referred for secondary review.
“Symptom or finding within 24h”	“Symptom or finding within 24h” applies to Initial reviews and Episode Day 1 reviews. Interventions, treatments, and medications are expected to be provided on the episode day they are specified in criteria unless otherwise noted such as “scheduled or performed within 24h.”
Medical practitioner orders initiated in the ED	Medical practitioner orders initiated in the emergency department and emergency department data can be used to meet criteria on Episode Day 1. Example: a patient presents to the emergency department with an O ₂ sat level of 88% (0.88). Oxygen therapy is initiated at 40% (0.40) and the patient is scheduled for admission. On admission, the patient has an O ₂ sat of 91% (0.91) and 40% (0.40) oxygen is ordered to be continued. Selection of criteria would be appropriate based on the O ₂ sat level of 88% (0.88) documented in the emergency department and the ongoing need for oxygen supplementation.
Medications	Excludes PO medications unless noted. When doing a review, a scheduled medication order can be applied to the criteria based on the reasonable assumption that the order will be followed. A PRN medication order cannot be applied toward the criteria because the reviewer cannot speculate how frequently the patient will require the medication. PRN medications that have been administered can be used when conducting a retrospective review if the frequency is supported within the criteria.

Definitions for interpreting criteria

Oxygen saturation (O₂ sat)	Oxygen saturation (O ₂ sat) measurements are based on room air readings unless the criteria state otherwise.
Vital signs	Vital signs are considered sustained findings when they are abnormal for two or more readings greater than 15 minutes apart or are lasting at least 15 minutes. This excludes an isolated reading, a transient abnormal measurement, or a finding that requires urgent treatment (e.g., severe hypoxia, ventricular arrhythmia, or hypotension). Application of criteria for a vital sign finding should be based on confirmatory measurements repeated at regular intervals. Events that are infrequent or vital sign abnormalities that have resolved with outpatient treatment are not considered to be sustained (e.g., the resolution of hypertension following anti-hypertensive therapy or the resolution of tachycardia following rehydration).
Within acceptable limits	Refers to a level or status that is deemed clinically appropriate by the medical practitioner or organization and is reflected in the documentation.
Baseline	“at baseline,” “> baseline,” or “< baseline,” refers to either the patient's baseline or a newly established baseline. In the absence of documentation, a patient's baseline status may be presumed to be in the range of normal values for the given criteria point.
Hospital acquired	Refers to a condition that developed during the patient's current hospitalization. Criteria referring to conditions that were acquired during the current hospitalization cannot be applied for patients who have been discharged to home or transferred to a post-acute facility.
Failed outpatient treatment	Refers to a lack of response to treatment received through the medical practitioner's office, outpatient clinic (e.g., urgent care), trial of home care services, or emergency department (if appropriate for the selected condition). These criteria can be applied when the patient fails to respond to a more intensive

treatment beyond the routine treatments or medications delivered for a chronic condition.

Administration of an intervention

When criteria include administration of an intervention prior to the decision to admit as part of the findings and the same intervention is also included as an ongoing intervention, the initial administration of the intervention given prior to the decision to admit cannot be used to satisfy both criteria points. For example, in the following criteria, patients must receive IV fluid or blood product transfusion prior to the decision to admit and must be receiving an additional blood product transfusion in order to select the criteria point blood product transfusion under the intervention header.

The screenshot shows a criteria selection interface with the following structure:

- OBSERVATION, Both:**
 - Gastrointestinal (GI) bleeding and ≥ 1 liter IV fluid or blood product transfusion prior to decision to admit and **Both:**
 - Coffee ground emesis, hematemesis, hematochezia, or melena
 - Hct $\geq 21\%$ (0.21) or Hb ≥ 7.0 g/dL(70 g/L)
- Intervention, $\geq$ One:**
 - Hct or Hb monitoring at least 2x/24h
 - Blood product transfusion

In this case, the initial treatment prior to the decision to admit did not resolve the patient's symptoms. The patient continues to require additional treatment and is therefore eligible for admission.

Responder-type criteria

Responder-type criteria indicate whether the patient is responding to treatment and if the current level of care is still appropriate. On the appropriate episode days, for some levels of care, criteria may include one or more responder-type criteria.

Responder types include:

- **Early responder:** Criteria in the Acute or Intermediate levels of care that indicate the patient is clinically stable in the last 12 hours (for adult/obstetric patients) or 24 hours (for pediatric patients). Discharge is expected on the day Early responder criteria are selected. These criteria differ from Responder. They do not contain complications or comorbid criteria. Patients responding earlier to treatment would not have complications or comorbid conditions that extend their length of stay. The  symbol indicates that the criteria are not met.
- **Responder:** Criteria in the Observation, Acute, or Intermediate levels of care, Responder criteria indicate the following:

Patient is clinically stable in the last 12 hours.
Includes:

- Adult patients at Observation, Acute, or Intermediate
- Pediatric patients at Observation
- Pediatric obstetric patients

OR Patient is clinically stable in the last 24 hours.
Includes:

- Pediatric patients at Acute or Intermediate

- Discharge is expected on the day Responder criteria are selected. The  symbol indicates that the criteria are not met.

In some subsets, multiple conditions are combined within the same section of Responder criteria. When conducting a review, the reviewer should select the clinical stability criteria related to the patient's primary or co-morbid condition. If more than one condition is active, all applicable clinical stability criteria should be selected.

- **Partial responder:** Criteria that indicate the patient is clinically appropriate at that level of care, for the designated episode day and condition.

- **Non-responder:** Criteria that indicate the patient may require continued stay; however, the episode days within the current subset and level of care have been exhausted. The  symbol indicates that the criteria are not met.

If documentation supports Non-responder criteria, conduct a review using the Extended Stay subset if redirection is provided or refer for secondary review.

Responder-type criteria notes: Responder and Non-responder days are generally based on Length of Stay (LOS) data. In most condition-specific subsets, Responder criteria are introduced on Episode Day 2 and are labeled “Early responder” until one day after the LOS 25th percentile. Going forward from the episode day in which responder criteria appear, there will always be Responder criteria and either Partial responder or Non-responder criteria. Non-responder days occur one day after the geometric mean length of stay (GMLOS).

Episode Days 2-X, 3-X, the Extended Stay subset, and some conditions at the Observation level of care include all three responder types on some episode days. Some of the early episode days within a subset may not contain any Responder criteria. Responder criteria will only be present if discharge is clinically expected or reasonable on that episode day. If an episode day does not contain Responder criteria and the patient is stable for discharge, the discharge should still occur.

Guidelines for applying Responder-type criteria

Keep the following in mind when applying Responder-type criteria:

When there are more than one responder type criteria in an episode day	When an episode day includes more than one responder type, only one can be met.
Order of applying responder-type criteria	Apply the Responder criteria first. If Responder criteria are not met, then apply Partial responder or Non-responder criteria.
When Responder criteria are met	Use the Discharge Screens as needed to assist in determining the most appropriate post-acute level of care: • The Discharge Screens are a resource tool and are available at any time during a review. • The Discharge Screens are organized from the least to most intensive level of care. • Referring to the Discharge Screens at the initiation of discharge planning is recommended. • There is no time requirement for Discharge Screens. • For a guide to planning a safe and effective transition to a post-acute level of care, refer to the InterQual[®] Transition Plan tool.

Guidelines for applying criteria in the General Surgical subset

If a patient has an ambulatory procedure and post-operative functional impairment that requires rehabilitation initiation within 24h or a complication that requires Observation	Use the criteria for Operative Day or Post-op Day 1 under the Observation level of care.
If a patient has complications that require treatment at a higher level of care	Apply criteria using the Intermediate or Critical level of care. For complications that are not included in the General Surgical subset, use the most appropriate condition-specific or general subset based on the patient's symptoms or findings. Example: criteria for stroke are in the Stroke subset. They are not a complication in the General Surgical subset.

Routine post-op stay vs post-op complication or comorbid condition

Criteria for routine post-op course should be used and exhausted prior to the application of complication or comorbid condition criteria.

Example: a patient who comes in for a moderate stay procedure should be reviewed using the routine post-op stay criteria until Episode Day 5, at which point the reviewer would select the appropriate complication or comorbid condition that drives the need for continued stay.

To review patients receiving a bone marrow or stem cell transplant

Use the Hematology/Oncology: Treatments subset.

Guidelines for applying criteria in the adult General Trauma subset

Criteria in the adult General Trauma subset address injuries that require staged operative procedures in addition to medical management (e.g., burns) or require an ambulatory procedure as part of their treatment plan (e.g., eye injuries).

If a patient requires continued stay

A different category may be used to evaluate the patient (e.g., general, respiratory, neurological), injury, or intervention depending on the need for continued hospitalization. Despite a change in the reason for continued stay, the review should be conducted for the next Episode Day, rather than starting from Episode Day 1, since the injury originated from the initial trauma.

If the patient sustains injury while hospitalized (e.g., fall from a bed)

Apply Episode Day 1 criteria since this is a new condition.



Follow the link for help on [what to do when a patient's condition changes, or the patient develops a new condition.](#)



Follow the link for help on [what to do when a patient is transferred to another facility.](#)

5. Take action based on review findings

Take action based on the review findings.

Review finding	Action
Initial review criteria are met	1. Approve recommended level of care. 2. Conduct Episode Day 1 review when full medication or intervention orders are available.
Episode Day criteria or Partial responder criteria are met	1. Approve level of care for that episode day. 2. Schedule next review.
Responder criteria are met	Prepare for discharge. Review Discharge Screens to determine appropriate post-acute level of care.
Non-responder criteria are met	Select Extended Stay subset, if applicable, and conduct a review or refer for secondary review.
Episode Day criteria, Responder, Partial responder, and Non-responder are not met	Obtain additional information from attending medical practitioner or other caregivers:

Review finding	Action
	- If additional information does not support criteria, discuss condition with medical practitioner. An alternate setting may be more appropriate for managing the patient's condition. - Facilitate transfer if medical practitioner is agreeable to alternate setting or level of care. - Refer for secondary review if medical practitioner does not agree.

Apply the CMS Two-Midnight Rule



Episode Day 1

The CMS Two-Midnight Rule generally requires that the medical record support a patient's need for hospital-level services **AND** that the patient has a hospital stay of at least two midnights.¹ Short-stay exceptions to this rule are defined below. Application of InterQual Criteria can help an organization comply with the medical necessity requirements of the two-midnight rule.

If your patient is affected by the CMS Two-Midnight Rule, follow these steps:

- Determine the need for hospital-based services by applying criteria.** If a patient meets criteria at any level of care, then they have met the medical necessity requirement of the rule.
- Determine whether the patient is appropriate for inpatient or observation status** depending on which level of care is met and the expected length of stay. Based on the presentation of the patient, evidence-based criteria such as InterQual can support the provider's expectation for length of stay.

If ...	Then ...
Care is expected to span two midnights, and the patient has met criteria at the Acute, Intermediate, or Critical level of care	Inpatient status may be appropriate. Note: In the event of an "Unforeseen Circumstance" per CMS resulting in a shorter stay than initially expected, a status of inpatient may still be appropriate in these situations: - Leaving against medical advice (AMA) - Unexpected death - Transfer of the patient - Unexpected clinical improvement - Election of hospice care
Care is expected to span less than two midnights but Acute, Intermediate, or Critical criteria are met	Initially assigning observation status may be appropriate. Note: CMS has defined the following exceptions as being appropriate for inpatient status, regardless of length of stay: - Mechanical Ventilation, established during current hospitalization - Surgical procedure or intervention found on the CMS Inpatient Only List

¹ Centers for Medicare & Medicaid Services, Title 42; Part 412; Subpart A - Admissions. 2023; Centers for Medicare & Medicaid Services, Title 42; Part 422; Subpart C-Benefits and Beneficiary Protections. 2023; Centers for Medicare & Medicaid Services, In: BFCC-QIO 2 Midnight Claim Review Guideline. 2022

If ...	Then ...
It is undecided whether care will span two midnights	The reviewer may use the level of care met as a guide to determine whether the patient is appropriate for inpatient or observation status. The criteria consider multiple factors, including clinical severity, comorbidity, and risk when differentiating between the Observation and Acute levels of care. These factors impact the length of stay. A patient who meets criteria at the Acute level of care can generally be expected to have a length of stay of greater than two midnights and thus may be assigned to inpatient status. Patients who meet criteria at the Observation level of care are more appropriate for observation status.



Episode Day 2

For patients who are in observation status and do not meet Observation responder criteria on Episode Day 2, apply Episode Day 2 criteria at the Observation, Acute, Intermediate, or Critical level of care in the appropriate subset to demonstrate the continued need for hospital-based services.

Note: For review of a surgical patient, Post-Operative Day 1 equates to Episode Day 2.



Episode Day 3

For patients who are in observation status beyond the second midnight (Episode Day 3) and who do not meet Observation responder criteria, apply Episode Day 3 inpatient criteria in the appropriate condition-specific or general subset.

Note: Patients who meet criteria at any of the inpatient levels of care based on medically necessary hospital-based services, and supported by clinical documentation, would be appropriate for inpatient status, while those who do not meet criteria would be appropriate for secondary review.

What to do when ...

To learn more about what to do when a particular situation arises during a review, follow the hyperlinks below.

[What to do when the patient's level of care does not match their physical location](#)

[What to do when a patient's condition changes or the patient develops a new condition](#)

[What to do when a patient is transferred](#)

What to do when a patient's physical location does not represent the level of care the patient is receiving

The patient is located at a place of care that is different from the assigned level of care

Action	Use the criteria set aligned with the level of care assignment.
Example	If a patient is in an Acute care bed, but is assigned Observation status, you should conduct a review using the Observation criteria.

An infant ≤ 28 days old is admitted to an Acute care facility

Action	Use the appropriate level of care within the Nursery subset that corresponds to the severity and intensity of services the infant is receiving, regardless of the location of care.
Example	An infant ≤ 28 days old who is admitted to the pediatric unit is still considered a neonate, so the Nursery criteria is appropriate to support medical necessity.

The patient has moved or needs to move to a higher level of care for the same diagnosis or condition (same subset)

Action	Use the higher level of care for the same episode day. The same process applies when the patient can step down to a lower level of care; conduct the review for the same episode day.
Example	A patient is receiving services for Heated, humidified HFNC at < or equal to 1L/min and is supported at NICU level II, on the 5th day of their stay, and their need for HFNC level increases, the patient would qualify for continued stay in NICU level III, criteria point Heated, humidified, HFNC > 1L/min, on that same episode day.

The setting has the capability to safely provide higher-level services

Action	Use the higher level of care criteria.
Example	If an Acute care unit has the capability to provide Intermediate care (e.g., continuous medications), Intermediate level of care criteria can be used to approve care delivery.

A facility does not have the lower level of care setting

Action	Use the lower level of care criteria to conduct the review at the higher level of care setting.
Example	If a facility does not have an Intermediate care unit and higher flow oxygen cannot be provided on an Acute care unit, then the Intermediate level of care criteria may be applied even though the patient is physically cared for in a Critical care unit.

The patient who is appropriate at the Acute, Intermediate, or Critical level of care also meets criteria at a lower level of care

Action	Apply the higher level of care when clinical documentation supports the need for services at a higher level of care. This often occurs when an additional finding or risk factor impacts the severity of a condition or when a patient requiring a higher level of care is receiving the standard of care for their condition in addition to higher intensity treatments or interventions.
Example	A pregnant patient at 28 weeks gestation who presents with a temp, tachycardia (100bpm), flank pain, a positive UA (with a culture pending), and is being treated with IVF and an anti-infective would meet criteria in the Observation and Acute levels of care. In this scenario, the patient should be managed at the Acute level of care due to the co-morbid condition of pregnancy.
Exception	The exception to this rule occurs when Observation and Acute Partial responder criteria are identical on Episode Day 2. In this case, the review should be conducted using the same level of care that was met on Episode Day 1.

What to do when a patient's condition changes or the patient develops a new condition

When the patient develops a new condition that is identified as the clinical driver for continued stay, determine whether criteria for the new condition exists in the current subset.

The patient's new condition is not specified in the criteria

Action	Look for criteria based on what is driving the patient's need for continued stay.
Example	You can't find criteria for pneumonia in the Extended Stay subset. You should select criteria under either "Infection" or "Respiratory." For conditions, diagnoses, or symptoms not addressed in the Extended Stay subset, secondary review is appropriate.

The patient's new condition necessitates continued stay

Action	Refer to Episode Day 1 or Operative Day of the most appropriate subset.
Example	A patient develops cellulitis during the extended hospitalization. The reviewer should refer to the Infection: Cellulitis subset, Episode Day 1. A patient requires surgical intervention during the extended hospitalization. The reviewer should refer to the General Surgical subset.

The patient's new condition is included in the current subset

Action	Remain in this subset and conduct a review using the current episode day.
Note	Commonly occurring complications and comorbidities related to a condition are incorporated into condition-specific subsets to allow for management of complex clinical scenarios and to support ease of use for the reviewer. Management of active comorbidities and commonly occurring complications is considered in the number of episode days included in each subset and the overall potential length of stay.
Example	A patient with COPD is admitted to the Acute level of care with an acute exacerbation. On Episode Day 3, the COPD exacerbation has resolved, but due to corticosteroid administration, the patient's comorbid diabetes mellitus is uncontrolled. Criteria for managing uncontrolled blood sugar is included in the COPD subset on Episode Day 3, Acute, Partial responder. In this scenario, the reviewer would complete a review using the criteria in the COPD subset.

The patient's new condition is included in the current subset, but is not a complication or comorbidity of the original condition

Action	Remain in this subset but return to Episode Day 1 to conduct the review.
Example	A patient is admitted for alcohol or drug intoxication (found in the General Medical subset) and on Episode Day 3 develops hepatic encephalopathy, you should return to Episode Day 1 within the General Medical subset and conduct a review.

The patient's new condition is not included in the current subset

Action	Go to the appropriate subset based on the patient's clinical findings and conduct the review for Episode Day 1.
Example	A patient has a medical condition and during the episode of care requires surgery, conduct a review using the General Surgical subset.

The patient's condition changes and you are redirected to a more clinically appropriate subset

Action	Go to the subset to which you are redirected.
Example	A patient who presents with a deep vein thrombosis (DVT) is found to have a pulmonary embolism (PE) on Episode Day 2. PE is now the patient's primary condition, and the reviewer should use Episode Day 1 in the Pulmonary Embolism subset.

What to do when a patient is transferred to another facility

The patient is transferred to a new facility

Action	If criteria were met at the transferring facility, then it is not necessary for the accepting facility to conduct an Episode Day 1 review. The accepting facility should conduct the next review on the appropriate episode day.
Example	A patient was admitted with a STEMI and the following day they are transferred to another facility. In this case, the accepting facility would conduct a review for Episode Day 2.

It is not known if criteria were met at the transferring facility

Action	If it is not known if criteria were met at the transferring facility, do one of the following: - Accepting facility could conduct a retrospective Episode Day 1 review by applying information found in the transfer summary, report information, or other documentation sent from the original hospital. The review would be for the date of admission to the transferring (original) facility. - Conduct an Episode Day 1 review for the day of transfer to the new facility. In some cases, a transfer is done due to worsening symptoms or lack of improvement, and it may be possible to conduct an Episode Day 1 review based on these new or worsening symptoms. - Refer the case for secondary review.
Example	A patient is admitted to a Critical Access Hospital (CAH) with COPD on Episode Day 1. The patient's symptoms exacerbated necessitating a transfer to a larger hospital on Episode Day 3. The reviewer can apply criteria in the COPD subset on Episode Day 3 at the higher level of care, if information is available. If information is not available, the reviewer should conduct the review on Episode Day 1.

Secondary review

Secondary review determines the appropriateness of the current or proposed level of care when it is not supported by criteria on primary review. Organizational policy should dictate the extent to which secondary review is performed to render a review outcome. A secondary review may be completed by a supervisor, medical practitioner, subject matter expert, or designated clinician. A medical practitioner **is not required** to perform a secondary review. However, some regulatory requirements specify that only a physician can issue a final non-determination (denial) decision.

- A secondary review is appropriate when:
- Review criteria are not met.
- Criteria are met and there is a concern about the level of care based on the complexity of the patient's condition.
- There are questions about the quality of care.
- As identified by the organization.
- There is a discrepancy in review outcomes between payers and providers.

A secondary review answers questions such as:

- Does the patient require admission or continued stay at the current level of care?
- Are there considerations beyond the primary review criteria to be considered?
 - Does the patient require this level of care?
 - What are the treatment options?
 - Is there a quality-of-care question?
 - Should a specialist evaluate this case?

Steps for conducting a secondary review

1. Determine medical necessity by reviewing the medical record, discussing the case with members of the interdisciplinary team (e.g., nursing staff, the discharge planner, therapists, and the attending medical practitioner), and applying clinical knowledge and judgment.

The secondary reviewer may consider other factors in the determination such as:

- Age
- Co-morbid conditions (e.g., medical, substance use, and psychiatric disorders)
- Presence of a caregiver, life stressors, or safe home environment
- Social determinants of health (such as low health literacy and racial disparities)
- Treatment response and patient engagement
- History of non-compliance and health outcomes
- Other organization – or patient-specific factors

Note: The secondary reviewer is not required to, but may choose to, apply InterQual Criteria when conducting a secondary review. Best practice includes reviewing the results of the primary review to ensure internal agreement in criteria application.

The primary reviewer may assist the secondary review process by including the outcome of the primary review and documentation.

2. Determine the review outcome:
 - If the secondary reviewer agrees with the existing level of care, approve the level of care.

- If the secondary reviewer does not agree with the existing level of care, they discuss the alternate level of care options for this patient with the attending medical practitioner:
 - If the attending medical practitioner agrees with the secondary reviewer, facilitate the transfer to the alternate setting or level of care, if available.
 - If the attending medical practitioner does not agree with the secondary reviewer, initiate action as indicated by organizational policy.
3. Document the review outcome.

Additional information

Utilization Benchmarks

Length of Stay—Benchmark length of stay (LOS) targets are included in Acute Adult and Pediatric Criteria for most conditions. The InterQual benchmark length of stay values are derived from a set of deidentified claims data, representing millions of patient discharges and have been statistically validated. Values represent geometric mean length of stay for a specific condition and are based on ICD claims including co-morbidities. For conditions with more than one ICD code (e.g., NSTEMI), the LOS represents a weighted average based on the number of claims for each included ICD code. Additionally, in the Adult content, the Centers for Medicare & Medicaid Services Medicare Severity Diagnosis Related Groups Geometric Mean Length of Stay (CMS MS-DRG GMLOS) is provided.

The LOS value provides a benchmark that is not intended to serve as a treatment limit or substitute for clinical judgment but can support medical directors, utilization leaders, and other hospital and health plan professionals understand normal ranges for current practice.

Percent Paid as Observation—Percent paid as Observation data has been included in Acute Adult and Pediatric Criteria for most conditions. This data indicates the final payment status as a percentage for patients hospitalized with the listed condition. The data provided do not necessarily correspond to the status assigned at admission. This data is intended to provide national benchmarks to facilitate efficient management of a patient to benchmark status. It is not intended to replace criteria application when determining a level of care.

InterQual® Transition Plan Tool

The Transition Plan tool is a guideline to assist in planning for a safe transition to the most appropriate post-acute level of care. Reviewers are encouraged to begin using the Transition Plan tool at the time of admission. The Transition Plan:

- Is **not** a required part of the review process
- Outlines interventions necessary to ensure continuity of quality care
- Identifies patients who are at high risk for readmission
- Provides a framework for identifying discharge needs

Care Management Information

Available for select subsets, Care Management Information outlines the expected clinical progress of the condition and provides suggestions for managing a patient if there are barriers to clinical progression. Care facilitation to the next appropriate level of care is also included. In some subsets, a section for Admission Considerations has been included to identify the types of clinical findings/information that may be used to complete an admission review.

Gender considerations

InterQual® content contains references to patient sex or gender. Depending on the context, these references may refer to either genotype or phenotype. At the individual patient level, a variety of factors, including but not limited to gender identity and gender affirmation via surgery or hormonal treatment, may affect the applicability of some InterQual Criteria. This occurs most often with genetic testing and some procedures that require the presence of specific anatomy. For the purpose of these criteria, references to Male and Female are based on sex assigned at birth, unless otherwise defined. InterQual users should carefully consider patient genotype and anatomy when appropriate.

Variance days

A “variance day” is a day of care at a higher level of care than is necessary based on the review. When Discharge Screens are met and a lower level of care is appropriate, but unavailable, the reviewer should:

- Indicate the reason the patient has not been transferred.
- Assign a level of care that represents the appropriate alternate level of care, had it been available.
- Document the number of days (referred to as variance days) used at a specific level of care when a less intensive level is appropriate.
- Discuss the case with a secondary reviewer and document the review decision.